

# Patient A 500-Year Hardening Plan

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Classification: Alias-only, privacy-preserving

Subject: Patient A

## Executive Intent

This plan establishes a durable, evidence-first hardening doctrine for Patient A over a 500-year horizon. It is built to converge clinical safety, operational discipline, institutional memory, and anti-drift governance.

## Convergence Doctrine

- Fail closed on safety-critical uncertainty.
- Require provenance for every consequential decision.
- Encode controls so future teams can verify, not guess.
- Preserve alias-only identity handling in all artifacts.

## 12-Pronged Hardening System

1. Identity integrity and consent continuity.
2. Evidence chain and source provenance.
3. Clinical risk gating and contraindication controls.
4. Data minimization and compartment boundaries.
5. Tool/model trust boundaries and escalation.
6. Change control and release governance.
7. Incident response and rollback readiness.
8. Financial continuity and mission funding resilience.
9. Legal/regulatory adaptation layer.
10. Human oversight, role rotation, succession.
11. Infrastructure resilience and disaster recovery.
12. Anti-corruption, anti-drift, long-memory archives.

## Epoch Roadmap

### Epoch 0 (Years 0-2): Baseline Stabilization

- Stand up minimum viable controls for all 12 prongs.
- Establish red-line safety gates and stop conditions.
- Start monthly evidence attestations and quarterly hardening reviews.

### Epoch 1 (Years 3-10): Institutional Hardening

- Convert process knowledge into auditable control records.
- Add independent review loops and conflict-of-interest checks.
- Enforce release gates tied to evidence completeness.

### Epoch 2 (Years 10-50): Succession and Interoperability

- Formalize handoff doctrine across generations of operators.
- Maintain compatibility layers for evolving tools and standards.
- Preserve testable policy intent through schema-version governance.

### Epoch 3 (Years 50-500): Civilizational Durability

- Maintain low-assumption archives readable across eras.
- Re-validate core safety axioms at fixed interval cadences.

- Require periodic doctrine renewal without losing provenance.

## **Gate Model (Per Epoch)**

Each epoch gate must include:

- Entry criteria
- Exit criteria
- Evidence bundle
- KPI band
- Failure triggers
- Escalation owner
- Recovery playbook

Progress cannot advance when safety evidence is incomplete.

## **Minimum KPI Spine**

- Evidence coverage ratio
- Safety gate pass rate
- Median detection-to-escalation time
- Incident recurrence rate
- Control drift index
- Succession readiness score

## **Risks and Countermeasures**

- Risk: Overconfidence from incomplete evidence.
- Countermeasure: Automatic fail-closed gate with mandatory review.
- Risk: Control drift across operator turnover.
- Countermeasure: Versioned doctrine + attested handoffs.
- Risk: Institutional memory decay.
- Countermeasure: Long-memory archive format with checksum verification.

## **Operating Cadence**

- Monthly: control health review.
- Quarterly: convergence audit and gate re-certification.
- Annual: full doctrine stress test and renewal attestation.
- Every 5 years: succession simulation exercise.

## **Founder Close**

This is a persistence strategy, not a one-time document.

The core rule is simple: truth before speed, safety before narrative, evidence before confidence.